

SentinelRx

Evidence-grounded medication-transition review

Rules flag discrepancies. Evidence checks gate findings. The clinician decides.

01 / REVIEW

Potential omissions, additions, duplicate records and dose-text changes across supplied admission and discharge lists.

02 / CHECK EVIDENCE

Separate deterministic gate checks unique source references, medication identity, transition roles and the discrepancy predicate.

03 / KNOW THE LIMIT

VERIFIED means bundle-local checks passed. It does not establish clinical correctness. Ambiguous evidence triggers ABSTAIN.

Reproducible software evaluation

120 regression cases: F1 1.000 | set-only baseline F1 0.600
90 generated failure cases: exact findings 1.000, abstention 1.000, crashes 0
32 authored perturbations: exact findings 1.000, abstention 1.000, crashes 0

Inspect the working demo

Safety Review / Medication Timeline
Evidence Provenance / Counterfactual Lab
Evaluation + missing-identity scenario

Synthetic development tests only. Repeated templates and authored perturbations are not held-out or clinical validation.

Scope: active MedicationRequest/MedicationStatement; explicit transition tags; coded identity. No AI/LLM layer, general FHIR validation, prescribing or semantic dose equivalence. Absence assumes supplied lists are complete.

Prior work: same problem family and safety principles as FHIRGuard. This separate implementation focuses on interactive review, evidence states and counterfactual testing; the underlying concept is not claimed as new.